

The Global Burden of Mental Illness

Prevalence, Impact, and the Treatment Gap

Global data · 2023 · Source: Our World in Data / IHME Global Burden of Disease

A data-driven overview of global mental health conditions, their disease burden, and the persistent gap between need and treatment — tailored

Hundreds of Millions Affected

1 in 3 women and 1 in 5 men face major depression in their lifetime.

WOMEN

1 in 3

will experience major depression
in their lifetime

MEN

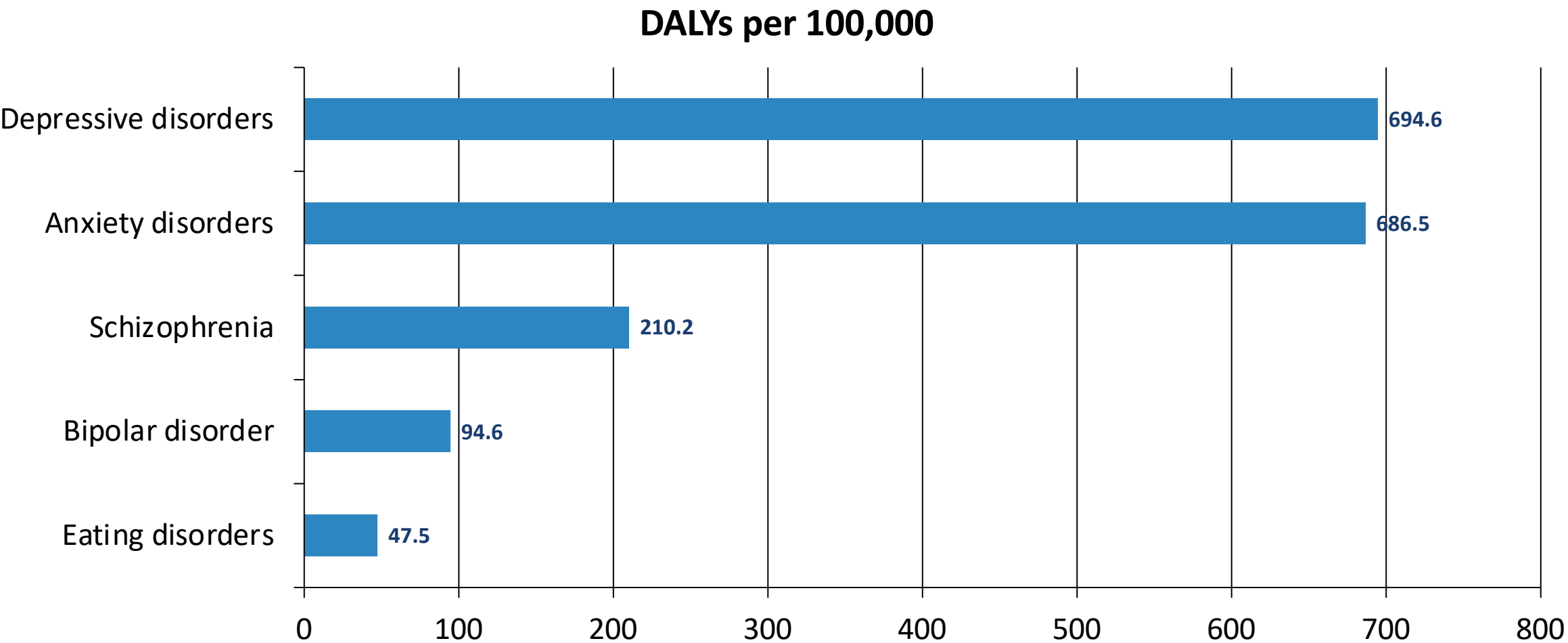
1 in 5

will experience major depression
in their lifetime

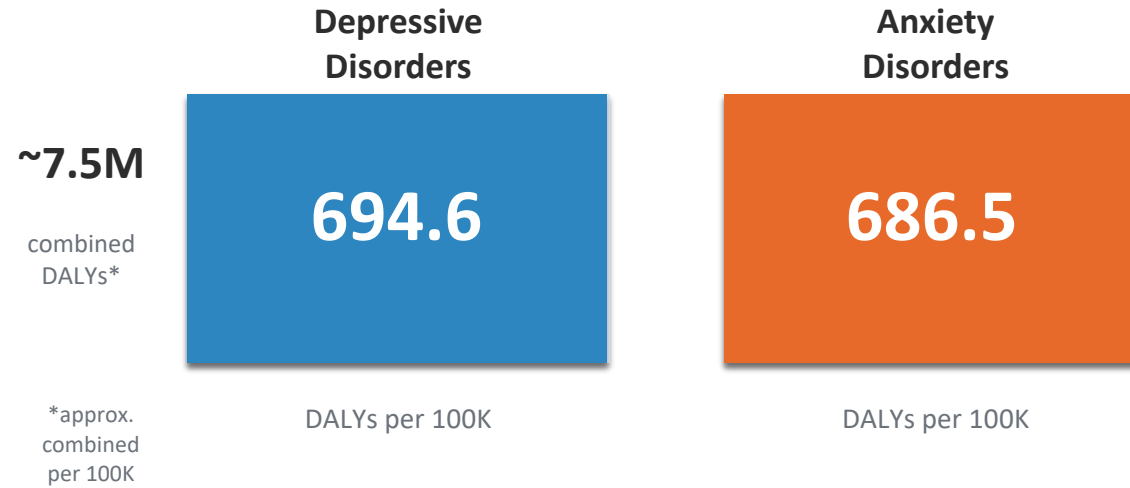
Mental health conditions span a wide spectrum — from highly prevalent disorders like depression and anxiety, to less common but severely disabling

Depression and Anxiety Dominate Disease Burden

DALYs per 100,000 people — World, 2023



Depression and Anxiety Carry Nearly Equal Global Burden



Together, depression and anxiety account for roughly 80% of total mental health DALYs across these five categories — dwarfing all other mental illnesses.

Source: IHME, Global Burden of Disease (2025)

Schizophrenia and Bipolar: Lower Prevalence, Severe Individual Impact

Schizophrenia

210.2

DALYs per 100K



Affects a smaller population but causes severe disability across the lifespan. R

Bipolar Disorder

94.6

DALYs per 100K



Characterized by extreme mood swings affecting millions worldwide. Individua

For reference: depressive disorders generate 694.6 DALYs per 100K — over 3× schizophrenia's rate.

Source: IHME, Global Burden of Disease (2025) · Our World in Data

Treatment Is Available but Often Lacking or Poor Quality

Mental illnesses are treatable and their impact can be reduced. Yet despite the availability of effective interventions, significant barriers prevent people

S

STIGMA

Many feel uncomfortable disclosing symptoms to healthcare professionals or people

A

ACCESS

Treatment is often lacking, especially in low- and middle-income countries.

Q

QUALITY

Even where treatment exists, it may be of poor quality or not evidence-based.

U

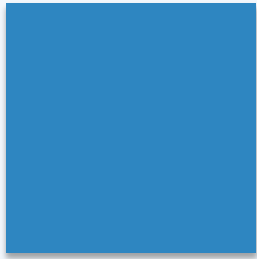
UNDERREPORTING

Discomfort with disclosure makes it difficult to estimate true prevalence — actual burden

Good data is essential to understand how, when, and why mental illnesses occur — and how they can be treated effectively.

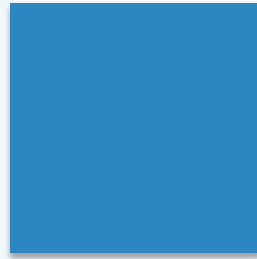
How People Cope: From Medication to Faith to Family

Global survey data reveals wide variation in how people deal with anxiety or depression.



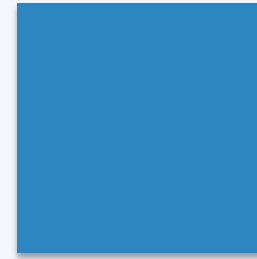
Prescribed Medication

Common in higher-income settings where healthcare



Religious / Spiritual Activities

Prevalent across many regions worldwide — faiths-based



Talking to Friends/Family

A widely reported mechanism across cultures — social

Policy interventions must account for these differences — responses shaped by cultural, economic, and healthcare system factors require a culturally sensitive approach

Key Takeaways: Evidence-Based Policy Priorities

01

Depression and anxiety together represent the overwhelming majority of mental health DALYs — each generating ~690 per 100,000, nearly e

02

Schizophrenia and bipolar carry disproportionate individual burden relative to their prevalence, demanding specialized care systems.

03

Treatment exists but access and quality remain inadequate globally — a large treatment gap persists across income settings.

04

Stigma suppresses both help-seeking and accurate data, meaning the true burden of mental illness is likely underestimated.

05

Policy should prioritize scaling evidence-based interventions for common disorders while maintaining specialized care for severe conditions.